BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. The clinical team owns booking, result review and action on overdue tests. This is an educational map, not an automated appointment or result tracker.

Check	Before starting	Early cycles	Later cycles / transition	Next treatment / completion	Follow-up
Blood tests	FBC, kidney and liver profile	Repeat as clinically indicated; confirm the unit's pre-treatment test plan	Review testing needs at treatment changes	Follow active medicines and abnormalities	Follow unresolved abnormalities
Echo / heart	History / symptoms review	No routine TC-specific echo	Investigate if clinically indicated	Follow any cardiac concern	Individual plan
ECG	ECG only for clinical indications or interacting medicines; not a routine TC-cycle requirement.	Symptoms / risk-based repeat	Review if cardiac concern	ECG and echo answer different questions	Clinical indication
Cancer imaging	Record baseline extent as appropriate	Clinical response / symptoms review	Review new symptoms; this TC regimen is adjuvant	Arrange radiotherapy or further treatment if indicated	Planned breast imaging; no routine CT/PET
Markers / bone health	Tests selected for active medicines	No routine tumour-marker clearance	No routine blood tumour markers when well	DEXA if endocrine/bone-risk indication	Follow the endocrine and bone-health plan

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00250 v9 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · 2026

THE WHOLE TREATMENT JOURNEY

Docetaxel + cyclophosphamide (TC): treatment and checks

Adjuvant high-risk node-positive or node-negative operable breast cancer

Docetaxel and cyclophosphamide on Day 1 every 21 days for four to six cycles. The team confirms the planned course before treatment. Radiotherapy, when indicated, follows chemotherapy.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early cycles	Later cycles / transition	Next treatment / completion	Follow-up
Cancer medicines NCCP 00250	Agree aim, indication and medicines	Docetaxel + cyclophosphamide (TC) starts	Review response, toxicity and exposure	Complete planned course / next decision	Ongoing review / subsequent plan
Surgery EARLY	Adjuvant: surgery already completed	No automatic operation during cycles	Review postoperative recovery and symptoms	Surgery was completed before this adjuvant regimen	Pathology and recovery guide next steps
Radiotherapy EARLY	Assess indication	Usually after cytotoxic chemotherapy	Arrange sequential timing	Radiation team confirms start	Review effects / follow-up
Blood tests NCCP 00250	FBC, kidney and liver profile	Repeat as clinically indicated; confirm the unit's pre-treatment test plan	Review testing needs at treatment changes	Follow active medicines and abnormalities	Follow unresolved abnormalities
Echo / heart NCCP 00250 · ESC	History / symptoms review	No routine TC-specific echo	Investigate if clinically indicated	Follow any cardiac concern	Individual plan
ECG NCCP 00250	ECG only for clinical indications or interacting medicines; not a routine TC-cycle requirement.	Symptoms / risk-based repeat	Review if cardiac concern	ECG and echo answer different questions	Clinical indication
Cancer imaging NCCP 00250	Record baseline extent as appropriate	Clinical response / symptoms review	Review new symptoms; this TC regimen is adjuvant	Arrange radiotherapy or further treatment if indicated	Planned breast imaging; no routine CT/PET
Markers / bone health NCCP 00250	Tests selected for active medicines	No routine tumour-marker clearance	No routine blood tumour markers when well	DEXA if endocrine/bone-risk indication	Follow the endocrine and bone-health plan
Endocrine / bone BONE · EARLY	Confirm hormone receptors	Plan endocrine therapy when appropriate	Separate from this chemotherapy prescription	Start/continue per MDT; DEXA when indicated	Risk-based bone review

What each check is for

1 Before first dose

Baseline

Baseline blood count, kidney and liver tests. The team arranges repeat testing as clinically indicated and confirms which results are needed before treatment.

Ask / act: Confirm tests and consent before prescribing.

Sources: NCCP 00250

2 Heart-monitoring plan

Baseline → cycles → after treatment

TC contains no anthracycline or HER2-targeted antibody. There is no routine three-month echo programme for TC alone; arrange cardiac tests for symptoms, prior exposure or other clinical indications.

Ask / act: Name the team responsible for booking and reviewing results.

Sources: NCCP 00250 · ESC

3 Treatment-phase review

Before each transition

Docetaxel and cyclophosphamide on Day 1 every 21 days for four to six cycles. The team confirms the planned course before treatment. Radiotherapy, when indicated, follows chemotherapy.

Ask / act: Confirm active medicines, support script and the next test checkpoint.

Sources: NCCP 00250

4 Response and follow-up

Planned review points

For early breast cancer, breast imaging and clinical review follow the agreed pathway. Routine CT/PET scans and blood tumour markers are not used to look for distant recurrence when there are no symptoms or clinical concerns.

Ask / act: Review symptoms and imaging together; do not change treatment on tumour markers alone.

Sources: NCCP 00250

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-23 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.17